جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : الشيماء عبدالخالق ابراهيم --

المحافظة : الجيزه **الحي / المدينة :** الهرم **العنوان :** 22 ش الاهرام - مساكن كفر الجبل الهرم

رقم البطاقة : 28010091400881 **رقم التليفون :** 01143436699 **تاريخ تسجيل الإستمارة :** 15-10-2026

العمر(تاريخ الميلاد) : 09-10-1980 **المهنة :** بدون عمل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي
التوقيع:(----------)

المقر بما فيه
التوقيع:(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي بيمبروليزوماب وريدي [14335] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: الشيماء عبدالخالق ابراهيم --
National ID: 28010091400881

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Pembrolizumab IV

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين


جمهورية مصر العربية
بطاقة تحقيق الشخصية
اميمه
فتحى احمد حبيب
كفر الشيخ خليل
مركز شبين الكوم - المنوفية
٢٧٧ ٠٣ ٢٩ ١٧ ٠٠٢ ٦١
KT9090003


٢٧٧٠٣٢٩١٧٠٠٢٦١
٢٠٢٤/٠٩
حاصله على دبلوم صناعه
انثى مسلمة متزوجة
أشرف محمد عبدالسلام القرشى
١٢
البطاقة سارية حتى ٢٠٣٨/٠٩/٢٤

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

05/13/2026

MDT FORM

Name:	Oumaima Ftea Ayman Hableb	MRN:	8074
Age:	49 Y	ID:	27703291700261
Menopausal Status:	premanop	Phone:	01008577482
NRS SCORE:	1	Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

BX: idc2

ER 5/8 PR 5/8 HER 3+ KI67 40%

Sonomammography:

Mels Workup:

History:

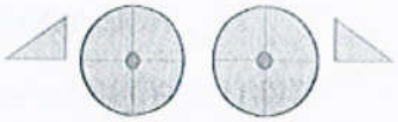
48 y prem left breast cancer ct4b n2 mo idc 2 er5/8 pr5/8 her2 3+ rec nact 8 cycles taxotere carboplatin 3 phesgo on 19/4/2026 left mrm invasive mucinous carcinoma yptl ypn1 3/16 for mdt

Recommendations:

MDT Outcome & Plan

For Kadcyle (complete 1y)
PORT to CW and LNs
adj ET AI Femara + Zoladex

R L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Name: الشيماء عبدالخالق ابراهيم السيسى	
MR. : 11241	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		**رقم طبي:**	

نتشرف بأن نوضح فيما يلي تقريرا عن حالتها

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انها

The patient was diagnosed with triple-negative breast cancer (TNBC), invasive ductal carcinoma grade II, estrogen receptor 0/8, progesterone receptor 0/8, HER2 0, Ki-67 70 %. Contrast-enhanced spectral mammography (CESM) demonstrated a heterogeneous right upper-outer quadrant mass measuring 4.6 × 3 cm with satellite nodules 1.9 cm anteriorly, overall area 5 cm, and a single level 1 pathologic lymph node 15 × 20 mm. Staging work-up with CT chest/abdomen/pelvis and bone scan was negative. Multidisciplinary team (MDT) recommendations included port-a-cath insertion, BRCA testing, clipping of both masses, and initiation of neoadjuvant systemic chemotherapy (NSCT) consisting of four cycles of pembrolizumab plus weekly taxol and carboplatin for 12 weeks, followed by pembrolizumab plus AC for four cycles, and subsequent right wide-local excision (WLE) with sentinel lymph node biopsy (SLNBx).

Pembrolizumab (Keytruda) approval by the Ministry of Health was pending; chemotherapy was to be started in the interim.

For cycle 1 week 1 (c1w1) of taxol-carboplatin, laboratory results were pending for weeks 2 and 3, and the pharmacy was to hold the regimen until pembrolizumab approval.

Pharmacy documentation recorded the patient's weight at 101 kg, height 165 cm, and surface area 2 m² for week 1 taxol 150 mg and carboplatin AUC 2.

The MDT on 30 June 2026 reiterated the plan: port-a-cath insertion, BRCA testing, clipping of both masses, start NSCT with four cycles of pembrolizumab plus weekly taxol-carboplatin for 12 weeks, then pembrolizumab plus AC for four cycles, followed by right WLE and SLNBx. Keytruda was pending; chemotherapy was scheduled for weeks 2, 3, and 4.

On 24 August 2026 the patient presented for pharmacy pick-up for week 3; BRCA testing was completed and showed no pathogenic variant.

Pharmacy note for week 3 confirmed taxol 150 mg and carboplatin AUC 2.

On 31 August 2026 the patient's comorbidities included diabetes mellitus, hypertension, and a right breast cancer classified as cT3 N1 M0. Imaging showed mild rib uptake, bilateral ovarian cysts pending MRI correlation, and a right upper-outer quadrant mass 4 × 6 cm with a right axillary lymph node 1 × 1 cm. Liver function tests were stable (ALT 54 U/L). BRCA1/2 testing remained negative. Clips were placed for TVUS assessment of ovarian cysts pending keytruda approval; MRI pelvis was scheduled for week 4. Taxol 150 mg and carboplatin AUC 2 were planned for week 4.

On 7 September 2026 the patient received week 2 taxol 150 mg and carboplatin AUC 2 as neoadjuvant therapy, and week 3 pharmacy handling was noted; liver support labs were normal and TVUS showed no ovarian cysts. Approval of keytruda remained pending.

تحريرا في ,

التاريخ :	20-09-2026	**الوقت :**	

الطبيب المعالج : خالد عبد العزيز محمد كمال

يعتمد ,

GUSTAVE ROUSSY

8074

Patient Name	: الأستاذه / اميمه فتحى احمد حبيب	Request No.	: 13005948
Patient No.	: 27703291700261	Request Date	: 19/04/2026
Path No.	: 1049/26	Verification Date	: 30/04/2026
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 49 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, post NACT.
Nature of the specimen: Left breast & Left axilla (MRM) (Post NACT)

Gross Examination:
- Procedure: Left MRM.

measured 22x20x6 cm with covering skin ellipse measured 22x15cm .
- Tumor bed Size: 7 x6 x4cm .
- Nipple & areola: Free of tumor cells .
- Distance from margins: 3 cm away from deep margin
- Axillary Lns: Fibrofatty tissue piece measured 6x5x2 cm entangling 11 nodules, largest 2x1 cm

BLOCK DESIGNATION
1-7: TBS
8: Nipple
9: Deep margin
10-13: ALNDs
Grossing Done by Dr.Heba El-Shafeey

- Microscopic examination:
- Histological type: Invasive mucinous carcinoma
- Actual tumour size: 1.5x1cm ,
- Tumor Cellularity: 10%
- Grade: 2 (differentiation3+ nuclear pleomorphism2 + mitosis1
- Ductal carcinoma in situ: Absent
- Lymphovascular emboli: Absent.
- Nipple and areola: free of tumor cells
- Axillary Lymph nodes: Positive for metastatic deposits in (3/16) LNs with no therapy effect

Verified by :
Dr Maha Guimie

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

GUSTAVE ROUSSY
INTERNATIONAL ACADEMY

Patient Name	: الأستاذه / اميمه فتحى احمد حبيب	Request No.	: 13005948
Patient No.	: 27703291700261	Request Date	: 19/04/2026
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Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 49 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

- Deep Margin: Free oftumor cells
- Histologic response to therapy: Pathologic partial response

DIAGNOSIS
Left breast, MRM (post NACT) :
- Residual invasive mucionous carcinoma grade 2
- Resection Margins :Free
- Nipple &areola: Free
- Axillary LNs: Positive for metastatic deposits in (3/16) LNs
- Pathological response to therapy :
- Pathologic partial response
- Stage : ypT1c , ypN1
- RCB : 3.44 , RCB class III

Signature
Prof Dr.Maha Guimei
Dr.Heba Hassan Adly

Verified by :
Dr Maha Guimie

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

NILE SCAN & LABS

مركز النيل للأشعة والتحاليل

Name : Omayma Fathy Ahmed Habib	Date : 28-12-2025 12:55
Branch : Nasrcity	Medical No. : 283532

Whole body PET/CT EXAMINATION:

Clinical History:

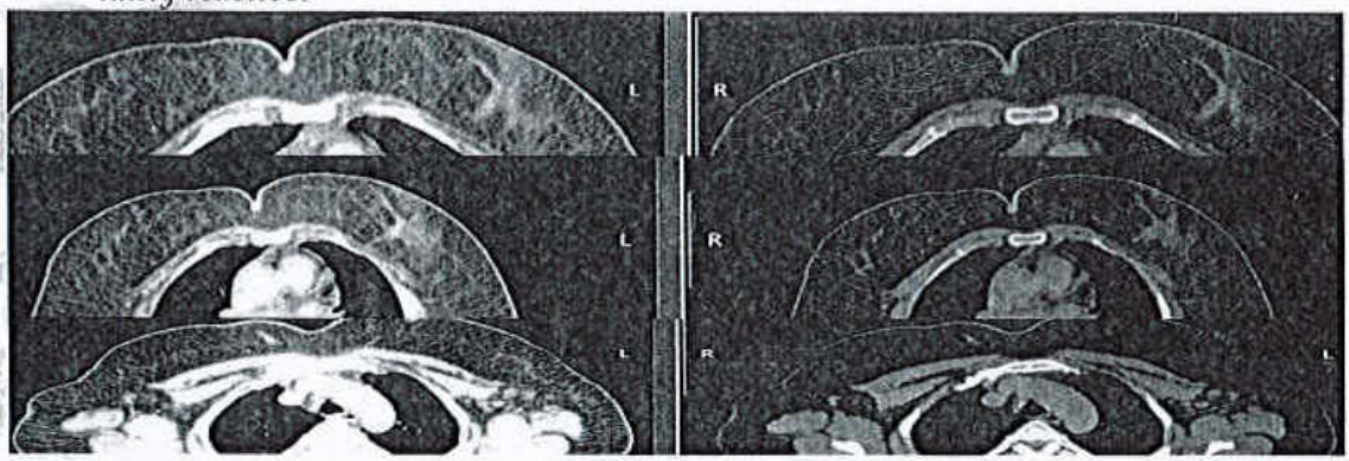
- *48 years old female patient presented with managed left breast cancer, received CTH sessions.*
- ***PET/CT requested for assessment.***

Technique:

- *For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed using GE Discovery IQ simultaneously and used for attenuation correction, anatomical localization and diagnosis. Max. variant of SUV; a Semi-quantitative analysis would be done for selected ROI.s and the normal threshold is < 2.5.*
- *Hepatic reference activity: SUVmax~4.*

Report:

- *Low grade FDG avid left breast ill-defined parenchymal density showing* ***SUVmax~1.***
- *Multiple non FDG avid bilateral axillary LNs are seen with preserved fatty hilum, likely reactive.*



Normal tracer uptake over brain, myocardium and renal system.

Head and Neck:

* *Normal diffuse brain activity with no hypermetabolic brain lesions.*
* *Physiologic FDG uptake is seen in the nasopharynx, oropharynx, salivary glands and larynx, showing normal appearance.*
* *Physiologic FDG uptake is seen in submandibular and parotid salivary glands, showing normal appearance.*
* *Otherwise, Normal size, enhancement & symmetrical appearance of thyroid glands with no FDG avid lesions detected.*
* *Normal appearance of the neck muscles & vessels.*
* *No hypermetabolic cervical lymphadenopathy detected.*
* *No FDG avid neck soft tissue mass lesions.*

Chest:

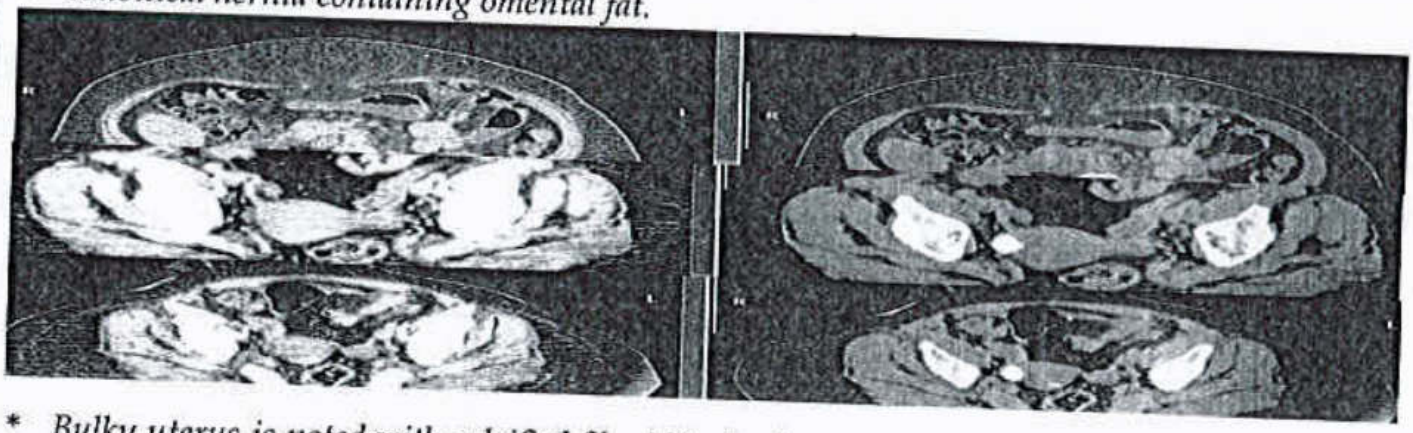
* *No hypermetabolic mediastinal or axillary lymphadenopathy.*
* *No FDG avid lung nodules or masses detected.*

Name : Omayma Fathy Ahmed Habib	Date : 28-12-2025 12:55
Branch : Nasrcity	Medical No. : 283532

* *No metabolically active pericardial or pleural lesions detected.*
* *Physiologic FDG uptake is noted in the heart, showing normal shape.*

Abdomen and Pelvis:

* *Physiologic diffuse FDG uptake is seen over average sized liver with homogenous parenchymatous texture. No FDG avid focal lesions detected.*
* *Physiologic diffuse FDG uptake is seen over average size spleen with homogenous parenchymal texture and no FDG avid focal lesions seen.*
* *Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.Physiologic FDG excretion is seen in the kidneys and bladder.*
* *Normal appearance of the pancreas, aorta, I.V.C and both supra-renals with no FDG avid lesions detected.*
* *No hypermetabolic abdominal or pelvic lymphadenopathy.*
* *No FDG avid ascites or abdominal collections noted.*
* *Umbilical hernia containing omental fat.*



* *Bulky uterus is noted with calcified fibroid indenting right distal ureter reflecting mild distal ureteric dilatation for further assessment.*

Musculoskeletal:

* *Normal FDG activity is seen over the axial skeleton.*
* *No focal active blastic or lytic bone lesions detected.*
* *Right sacro ileitis.*

Conclusion:

PETCT for a managed case of left breast cancer showing:

- ***Low grade FDG avid left breast ill-defined parenchymal density for US assessment.***
- ***No definite FDG avid active metastatic deposits.***

Thank you for referral

Mariam Ali, MSc

Walid Omar, MD.
Prof. of Nuclear Medicine

GUSTAVE ROUSSY INTERNATIONAL EGYPT

Patient Name	اميمة محمد احمد	Age	47
Sex	f	Special habits	
Family History		Comorbidity	
Surgical History		Telephone	01008577482
Menopausal Status	premenopausal	Marital Status	
ID	27703291700261	Medical ID	1837/2025
Date	10/8/2025		

Medical Report

Pathology: Bx-> IDC G II

ER 5/ 8

PR 5/ 8

HER2 +3

KI67 ?

S.M->
Lt breast UOQ malignant mass at 12 oclock 6cm from nipple measuring 2.7x 2.1 cm BIRADS 5
Rt breast BIRADS 1

MetsWorkup->
Bilateral lower lobes subcentimetric pulmonary nodules
Bilateral axillary lymphadenopathy
Myultipule enlarged abdominal LNs seen paraaortic, retrocaval and porta hepatitis. Largest measure 12x22 mm

Bone scan= free

Recommendations->

MDT DECISION	For Neoadjuvant cth TC HP followed by MRM

Name	د/ حنان عزت	د/ احمد نجيب
Signature		

GUSTAVE ROUSSY
INTERNATIONAL
EGYPT

Patient Name: Omayma Fat'hy Ahmed Habib	Date : 15/10/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 48 Year(s)
Medical ID: 1837/2025	Pat. ID: 110699

MULTISLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITH OUT CONTRAST

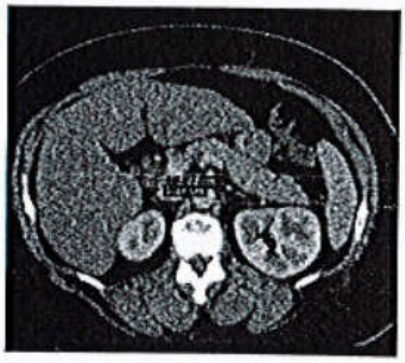
TECHNIQUE: Multi slice axial CT images of the chest, abdomen and pelvis were obtained followed by multiplanar reconstruction of the source images.

FINDINGS: compared to last study dated 26/7/2025

CECT CHEST:

- left breast architectural distortion .
- Bilateral lower lobes tiny subcentimetric pulmonary nodules.
- Right upper lobe, posterior segment shows mild pleural thickening.
- Bilateral lower lobes ground glass veiling.
- No appreciable changes regarding bilateral axillary lymph nodes are seen, largest one measures 16 x 11 mm compared to 14 x 12 mm on right side and measures 13 x 10 mm compared to 14 x 11 mm on left side.
- Sill noted Enlarged paratracheal and subcarinal lymph nodes, largest one measures about 6x4mm.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

CECT ABDOMEN AND PELVIS:



- The liver is average in size showing homogenous CT density with no focal lesions or biliary radicles dilatation.
- The gall bladder shows average wall thickness. Its cavity shows no dense stones.
- Normal CT appearance of the spleen, pancreas and both kidneys.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- Multiple enlarged abdominal lymph nodes seen paraaortic, retrocaval and porta hepatis, the largest one is the latter measures about 16 x 18 mm compared to 12 x 22 mm.
- No free collection.
- Supraumblical hernia defect measures about 4.3 cm contains fat.
- Uterine small calcific lesion.

THE SCANNED SKELETON:



- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.
- Right iliac bone diffuse sclerosis ..? sacroillitis? Suspicious area for follow up.
- **Opinion:**

Overall stationary course

DR.Reeham.Khader

The General Organization for Teaching Hospitals and Institutes
Ahmed Maher Teaching Hospital

المنيئة العامة للمستشفيات والمعاهد التعليمية
مستشفى أحمد ماهر التعليمي

قسم الباثولوجى

إسم المريض : اصفة المحى احمد حسن — السن : 48 سنه

محول من : (2025) (1171 - 2025)

تاريخ الحضور : 2025/05/28 — تاريخ الاستلام : 2025/06/01

NATURE OF SPECIMEN :

One paraffin block

PROCESS :

Ce-paraffin block sections are obtained for ER, PR, Her-2/neu, Ki67 using supersensitive prediluted antibodies with Bio-Vision IMPACT detection system employing peroxidase antigen retrieval and blocking enzymatic peroxidase and using DAB as chromogen using Benchmark GX automated immunostainer

MICROSCOPY :

Estrogen receptors: Moderate nuclear staining in 30% of tumor cells
Progesterone receptors: Moderate nuclear staining in 20% of tumor cells
Her-2/neu: Strong complete membranous staining in >10% of tumor cells
Ki67: Positive nuclear staining in 40% of tumor cells

DIAGNOSIS :

POSITIVE ESTROGEN RECEPTORS 5/8
POSITIVE PROGESTERONE RECEPTORS 5/8
POSITIVE HER-2/neu SCORE (3+)
KI67 POSITIVE NUCLEAR STAINING IN 40% OF TUMOR CELLS

RECOMMENDATION :

Prof. Dr. Basha Farouk, MD.

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient's Name:	**Al-Shaymaa Abdelkhalek Ibrahim El-Sisi**
Date:	**2026-09-07**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

Trans-vaginal ultrasound scanning revealed: -

- ***Uterus: -***
  - *RVF, bulky uterus measuring about: 9x5x4.5cm in its longitudinal, transverse & AP diameters respectively.*
  - *The myometrium shows homogenous texture with no focal lesion*
  - ***The endometrial line is 8mm.***
  - *The cervix is of average size with no focal lesion*

- ***Ovaries: -***
  - *Both ovaries are average size with left ovarian follicle measures 20x18mm*

- *No gross adnexal soft tissue masses or cysts.*
- *No free fluid in the Douglas pouch.*

Conclusion:

Bulky uterus

Dr/Nermeen ramadan.MD

اسم الفرع اللبيني - هرم
مدير الفرع د.عزه توفيق محمود عبد الواحد
مدير الوحده ا.د رانيا درويش
طبيب الوحده ا. د. علياء الدش

رئيس مجلس الادارة
ا. د. مؤمنه كامل

مدير المعامل
ا. د. هند الشربيني

PATIENT NAME			Registered	26-07-2026 21:20:38	Collected	26-07-2026 21:21:16
الشيماء عبدالخالق ابراهيم			Authenticated	12-08-2026 17:44:18	Printed	23-08-2026 17:36:15
Visit Number	Age	Gender	Referred By		Client ID	
70826511091	46 Year	Female	Prof : -		55452	

MOLECULAR BIOLOGY REPORT

BRCA 1.2 WHOLE GENE SEQUANCING / BLOOD (Astrazneca)

Result : No Pathogenic variants detected in BRCA1 or BRCA2 genes sequencing.

Background

- Hereditary Breast Cancer accounts for about 5-10% of female breast cancer and 4-40% of male breast cancer. The most common cause is mutations in the BRCA1 & BRCA2 genes which are tumor suppressor genes located on chromosomes 17 and 13 respectively.

- Pathogenic variants of these genes are inherited in an autosomal dominant manner and are associated with hereditary breast and ovarian cancer (HBOC) syndrome.
- HBOC syndrome increases the risk for breast (female and male), ovarian, fallopian tube, peritoneal, pancreatic, and prostate cancers. Additionally, BRCA2 carriers may be at increased risk for melanoma.

Method

- Genomic DNA was extracted from the patient's specimen. Libraries were prepared according to manufacturer's protocol then next-generation sequencing was performed on the Illumina MiSeq platform using CE-IVD BRaCA assay

- The assay analyzes sequencing of all exons and immediately adjacent intronic regions of BRCA1 & BRCA2 genes.

- Raw sequence data analysis, including base calling, demultiplexing, alignment to the hg19 human reference genome (Genome Reference Consortium GRCh37), and variant calling is done using standalone software. Variants are interpreted using publicly available databases such as Exac, GnomAD, dbSNP, ClinVar, Brca exchange. Variants are classified according to the American College of Medical Genetics and Genomics (ACMG) guidelines.

- Benign and likely benign variants are not included in this report.

- Variants of unknown significance classification may change over time.

Limitations

- Large rearrangements may not be detected, variants in regions with high GC content or pseudogenes may give false negative or false positive results.

Rania Darwish.

Dr. Rania Darwish
Prof of Clinical & Chemical Pathology,
Faculty of Medicine, Cairo University

رئيس مجلس الادارة
ا. د. مؤمنه كامل

مدير المعامل
ا. د. هند الشربيني

اسـم الفـرع اللبيني - هرم
مـدير الفـرع د.عزه توفيق محمود عبد الواحد
مـدير الوحده ا.د رانيا درويش
طبيب الوحده ا. د. علياء الدش

PATIENT NAME			Registered	26-07-2026 21:20:38	Collected	26-07-2026 21:21:16
الشيماء عبدالخالق ابراهيم			Authenticated	12-08-2026 17:44:18	Printed	23-08-2026 17:36:15
Visit Number	Age	Gender	Referred By		Client ID	
70826511091	46 Year	Female	Prof : -		55452	

- The test result is for clinical reference and clinicians should interpret it in the context of the patient's condition, clinical findings, family history, other lab investigations, and international guidelines for treatment.

References

- Yoshida, Reiko. "Hereditary breast and ovarian cancer (HBOC): Review of its molecular characteristics, screening, treatment, and prognosis." Breast Cancer 28.6 (2021): 1167-1180.

- Richards, Sue, et al. "Standards and guidelines for the interpretation of sequence variants: a joint consensus recommendation of the American College of Medical Genetics and Genomics and the Association for Molecular Pathology." Genetics in Medicine 17.5 (2015): 405-423.

Rania Darwish.

Dr. Rania Darwish
Prof of Clinical & Chemical Pathology,
Faculty of Medicine, Cairo University

Patient's Name:	**Al-Shaymaa Abdelkhalek Ibrahim El-Sisi**
Date:	**2026-09-07**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

Trans-vaginal ultrasound scanning revealed: -

- ***Uterus: -***
  - *RVF, bulky uterus measuring about: 9x5x4.5cm in its longitudinal, transverse & AP diameters respectively.*
  - *The myometrium shows homogenous texture with no focal lesion*
  - ***The endometrial line is 8mm.***
  - *The cervix is of average size with no focal lesion*

- ***Ovaries: -***
  - *Both ovaries are average size with left ovarian follicle measures 20x18mm*

- *No gross adnexal soft tissue masses or cysts.*
- *No free fluid in the Douglas pouch.*

Conclusion:

Bulky uterus

Dr/Nermeen ramadan.MD

اسم الفرع اللبيني - هرم
مدير الفرع د.عزه توفيق محمود عبد الواحد
مدير الوحده ا.د رانيا درويش
طبيب الوحده ا. د. علياء الدش

رئيس مجلس الادارة
ا. د. مؤمنه كامل

مدير المعامل
ا. د. هند الشربيني

PATIENT NAME			Registered	26-07-2026 21:20:38	Collected	26-07-2026 21:21:16
الشيماء عبدالخالق ابراهيم			Authenticated	12-08-2026 17:44:18	Printed	23-08-2026 17:36:15
Visit Number	Age	Gender	Referred By		Client ID	
70826511091	46 Year	Female	Prof : -		55452	

MOLECULAR BIOLOGY REPORT

BRCA 1.2 WHOLE GENE SEQUANCING / BLOOD (Astrazneca)

Result : No Pathogenic variants detected in BRCA1 or BRCA2 genes sequencing.

Background

- Hereditary Breast Cancer accounts for about 5-10% of female breast cancer and 4-40% of male breast cancer. The most common cause is mutations in the BRCA1 & BRCA2 genes which are tumor suppressor genes located on chromosomes 17 and 13 respectively.

- Pathogenic variants of these genes are inherited in an autosomal dominant manner and are associated with hereditary breast and ovarian cancer (HBOC) syndrome.
- HBOC syndrome increases the risk for breast (female and male), ovarian, fallopian tube, peritoneal, pancreatic, and prostate cancers. Additionally, BRCA2 carriers may be at increased risk for melanoma.

Method

- Genomic DNA was extracted from the patient's specimen. Libraries were prepared according to manufacturer's protocol then next-generation sequencing was performed on the Illumina MiSeq platform using CE-IVD BRaCA assay

- The assay analyzes sequencing of all exons and immediately adjacent intronic regions of BRCA1 & BRCA2 genes.

- Raw sequence data analysis, including base calling, demultiplexing, alignment to the hg19 human reference genome (Genome Reference Consortium GRCh37), and variant calling is done using standalone software. Variants are interpreted using publicly available databases such as Exac, GnomAD, dbSNP, ClinVar, Brca exchange. Variants are classified according to the American College of Medical Genetics and Genomics (ACMG) guidelines.

- Benign and likely benign variants are not included in this report.

- Variants of unknown significance classification may change over time.

Limitations

- Large rearrangements may not be detected, variants in regions with high GC content or pseudogenes may give false negative or false positive results.

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رئيس مجلس الادارة
ا. د. مؤمنه كامل

مدير المعامل
ا. د. هند الشربيني

اسـم الفـرع اللبيني - هرم
مـدير الفـرع د.عزه توفيق محمود عبد الواحد
مـدير الوحده ا.د رانيا درويش
طبيب الوحده ا. د. علياء الدش

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Visit Number	Age	Gender	Referred By		Client ID	
70826511091	46 Year	Female	Prof : -		55452	

- The test result is for clinical reference and clinicians should interpret it in the context of the patient's condition, clinical findings, family history, other lab investigations, and international guidelines for treatment.

References

- Yoshida, Reiko. "Hereditary breast and ovarian cancer (HBOC): Review of its molecular characteristics, screening, treatment, and prognosis." Breast Cancer 28.6 (2021): 1167-1180.

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National ID – Front & Back

جمهورية مصر العربية
بطاقة تحقيق الشخصية



الشيماء
عبدالخالق ابراهيم السيسى
٢٢ش الاهرام-مساكن كفر الجبل
الهرم - الجيزه

٢٨٠١٠٠٩١٤٠٠٨٨١

JB6652360

٢٨٠١٠٠٩١٤٠٠٨٨١ ٢٠٢١/١٠
حاصلة على دبلوم صناعى

أنثى مسلمة متزوجة
عونى حسن محمدراتب عبدالجواد
البطاقة سارية حتى ٢٠٢٨/١٠/١٨

[[DOCTYPE_OTHER_DETAILED_REPORT]]
detailed-report

جوستاف روسي
انترناشونال
مصر

إسم المريض ثلاثي: .....
رقم الملف: .....

تقريـــر طبـــى

السيد/ ..... إلى من يهمه الأمر .....

تحية طيبة وبعد،،

إيماء للطلب المقدم من ..... بشان تقرير طبي للمريض: ..... إلى السيد/ حسن كامل

بقسم..... رقم طبي .....

نتشرف بأن نوضح فيما يلي تقريرا عن حالته :

تاريخ وساعة الدخول..... تاريخ وساعة الخروج.....

وبعد الكشف والفحوصات والمتابعة وجد انه:

46 y old premenop RT Breast
CT3, N1, M0 BS Rib warm uptake
for CT chest
CT abd free
MRI pelvis free
IDC II Triple -ve
pt is on Neoadjuvant Taxol, Carboplat
No approval of Keytruda at ministry of health
Ready for Neoadjuvant Keytruda

تحريرا في

الوقت: ..... التاريخ : / /

الطبيب المعالج

.....

يعتمد،،

د/ رشا حمدي